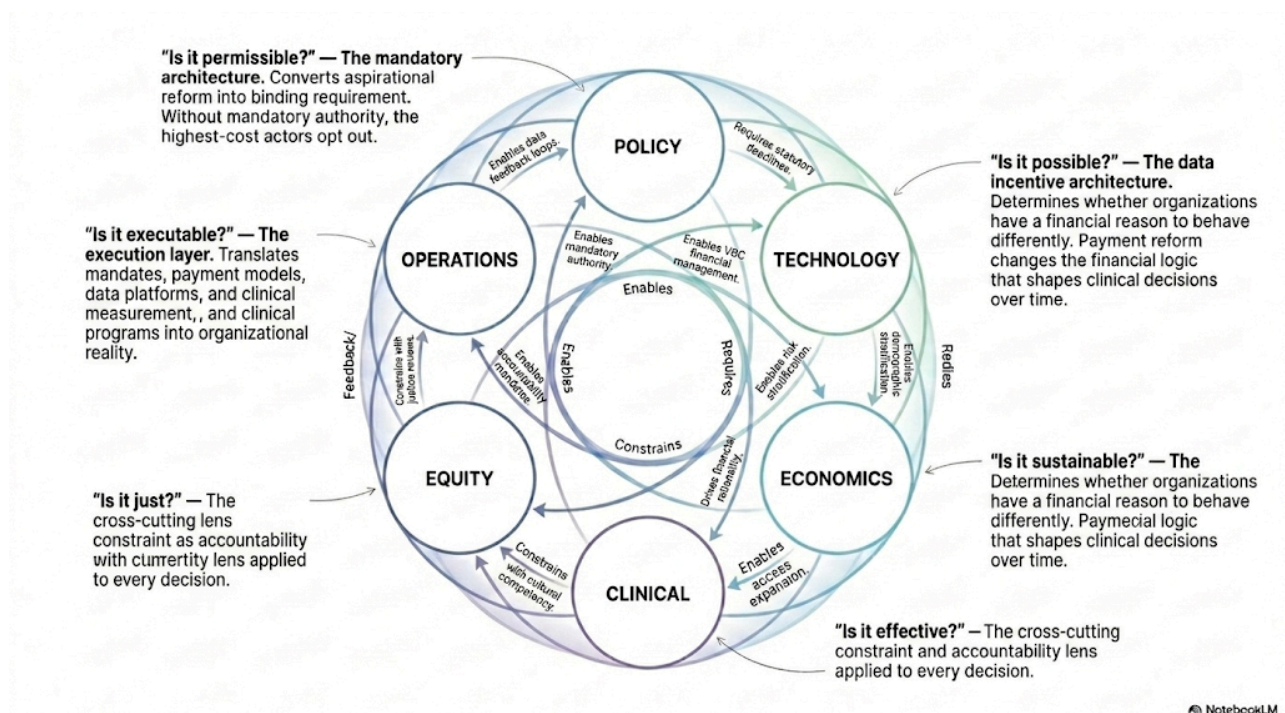


TRANSFORMING AMERICAN HEALTHCARE

A Six-Pillar Framework for System Transformation

Health Transformation Review

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PREFACE

Why This Book Exists

American healthcare is the most expensive, most administratively complex, and most inequitably distributed healthcare system in the developed world. This is not a new observation. It has been documented, analyzed, debated, and lamented for decades. What is new — or more precisely, what is newly urgent — is that the system is now visibly breaking under pressures that cannot be managed through incremental adjustment. The financial logic of fee-for-service payment, the demographic reality of an aging population, the exhaustion of a clinical workforce buried in administrative burden, and the structural inadequacy of a hospital network designed for a different century are converging into a crisis that requires systematic response, not marginal reform.

This book is a response to that crisis. It is designed for the healthcare leaders, policymakers, analysts, and students who must understand not just what is wrong with American healthcare but what a transformed system would look like, how to get there, and what the organizations executing that transformation need to know to succeed. It is simultaneously an academic text — grounded in evidence, rigorous in its analytical framework, honest about what is uncertain — and a practitioner's guide — organized around decisions, investments, contracts, and programs that real organizations must make in real time.

The book is organized around the six-pillar framework: Policy, Economics, Operations, Clinical, Equity, and Technology. This framework emerged from the conviction that healthcare transformation fails when it addresses any single pillar in isolation. Payment reform without clinical redesign produces financial savings that evaporate when care models do not change. Clinical quality improvement without payment reform produces better care that the financial system immediately discourages. Technology investment without operational readiness produces platforms that no one uses. Equity goals without structural SDOH investment produce aspirations without outcomes. The six pillars must move together — which means the people responsible for each pillar must understand all the others.

Vermont runs through this book as its primary teaching case. This requires explanation, because Vermont is a small state — 647,000 people, 14 hospitals, roughly the population of a mid-sized American city — and its direct policy relevance might seem limited. Vermont is in this book not because Vermont is large or economically powerful, but because Vermont is early. Early in its demographic aging. Early in its hospital financial fragility. Early in its recognition that voluntary payment reform cannot produce the structural change a system in crisis requires. And early in its willingness to legislate the structural responses — Acts 167 and 68, the AHEAD Model participation, the Rural Health Transformation Program — that other states will reach for when their own systems arrive at Vermont's current condition.

Vermont is also unusually transparent. The combination of a small state's intimacy, a strong public-sector analytical tradition, and an active regulatory body in the Green Mountain Care Board has produced a public record of healthcare system analysis — the Oliver Wyman Act 167 report, the GMCB's monthly hospital data, the AHS transformation reports, the AHEAD State Agreement — that gives this book a documentary foundation most states cannot match. Vermont's healthcare transformation is happening in public, with detailed data, and the lessons are directly transferable.

A word about what this book is not. It is not a policy brief for or against any particular political agenda. Healthcare transformation is contested terrain, and this book presents the evidence and the analytical frameworks that inform decisions without pretending those decisions are purely technical. The five goals of Vermont's Act 167 — reduce inefficiencies, lower costs, improve health outcomes, reduce health inequities, and increase access to essential services — are goals that virtually every person across the political spectrum would endorse. The means for achieving them are legitimately contested, and this book engages that contestation honestly.

It is also not a prediction. The transformations described in this book are underway but unfinished. The outcome of Vermont's experiment — whether the hospital system stabilizes or contracts, whether global budgets reduce costs or constrain access, whether the Agency of Human Services can execute the restructuring its mandate requires — is genuinely uncertain. This book provides the analytical framework for understanding what is happening and why, and for contributing to better decisions as the story unfolds. The decisions, and their consequences, belong to the people who make them.

April 2026

INTRODUCTION

|The Transformation Imperative

A System at the Breaking Point

On September 18, 2024, the Green Mountain Care Board of Vermont convened a public meeting at which Oliver Wyman Healthcare and Life Sciences Group presented the findings of a year-long, \$1 million statewide hospital systems analysis commissioned under Act 167 of 2022\ . The room was full. The findings were not surprising to anyone who had been paying attention — but they were stated with a precision and a comprehensiveness that made them impossible to dismiss.

Nine of Vermont's fourteen hospitals were reporting operating losses in 2023, with the worst reaching negative 8.9%. If current trends continued, thirteen of the fourteen would be reporting losses by 2028\ . To simply break even, the hospital system must overcome a five-year deficit ranging from a best-case estimate of 700 million to a more realistic projection of 2.4 billion, driven by skyrocketing labor and operating costs. Average premiums for silver exchange plans had increased 108% in six years, while median household income had grown 22%. Out-of-pocket maximums had more than doubled in five years. The working-age population was declining; the over-65 population was growing toward 30% of the state's total by 2040\ .

The Oliver Wyman consultants offered a three-imperative summary of what Vermont needed to do: build housing and fix transportation; pay hospitals with reference-based pricing and move to global budgets; and move all care possible out of hospitals. These were not incremental recommendations. They were structural prescriptions for a system that, absent structural change, was heading toward collapse.

Vermont is not an outlier. It is a preview.

The structural forces producing Vermont's crisis — demographic aging, hospital financial fragility, the unsustainability of commercial cross-subsidy, the failure of voluntary payment reform, the exhaustion of a workforce buried in administrative complexity — are present to varying degrees in every American state. Vermont faces them earlier and more acutely because of its demographics and its economic structure. But the states of the Northeast, the rural Midwest, and eventually the national system will arrive at Vermont's current condition on a lag of ten to twenty years. What Vermont is forced to solve now, the rest of the country will be forced to solve later.

This book is written in that context. It is not a retrospective analysis of a completed transformation. It is a real-time account of a transformation in progress, with all the uncertainty, difficulty, and instructiveness that implies.

What Transformation Actually Means

The word 'transformation' is used so frequently in healthcare that it has nearly lost its meaning. Press releases describe routine process improvements as transformational. Strategic plans invoke transformation as aspiration rather than commitment. This book uses the word in its precise sense: transformation is structural change that alters the fundamental incentives, relationships, and capabilities of a system, producing different behaviors and different outcomes rather than incremental improvements within the existing structure.

By this definition, most of what has been called healthcare transformation over the past fifteen years has not been transformational. Voluntary accountable care organization programs that high-cost providers can decline to join are not transformational. Shared savings models that reward hospitals for reductions in utilization they would have achieved anyway are not transformation. Electronic health record adoption that digitizes paper processes without changing care delivery is not transformation. Quality measurement initiatives that produce reports without affecting payment are not transformation.

Transformation, in the sense this book uses the term, looks like what Vermont is attempting: mandatory reference-based pricing that eliminates the ability of dominant hospitals to charge commercial payers three to four times Medicare rates; statutory global hospital budgets that change every hospital's financial incentive from maximizing volume to managing population health; a Statewide Health Care Delivery Strategic Plan that commits Vermont's Agency of Human Services to specific, measurable outcomes by specific dates; and capital investment through the Rural Health Transformation Program that builds the community infrastructure that the payment reform requires to produce population health improvement rather than just cost reduction.

Whether this constitutes successful transformation will be determined by what happens over the next five to ten years. What is already clear is that Vermont has moved further toward genuine structural change than any comparably sized system in recent American health policy history.

The Six-Pillar Framework: Why All Six Must Move Together

The central analytical contribution of this book is the six-pillar framework for healthcare transformation: Policy, Economics, Operations, Clinical, Equity, and Technology. The framework is not a checklist of things that should be done. It is an integrated analytical architecture for understanding why transformation attempts fail when they address any pillar in isolation.

The most common failure mode in healthcare transformation is single-pillar thinking. A payer that focuses exclusively on the Economics pillar — redesigning payment models — while leaving the Technology, Clinical, and Operations pillars unchanged produces payment reform that providers cannot operationalize. Hospitals without population health analytics infrastructure cannot manage global budgets. Clinicians without team-based care models cannot deliver the primary care access that makes reduced hospitalization possible. Administrators without administrative simplification cannot absorb the management complexity of value-based contracts on top of fee-for-service billing requirements.

Conversely, a technology vendor that focuses exclusively on the Technology pillar — building sophisticated health information exchange and population health platforms — while leaving the Economics and Operations pillars unchanged builds tools that no one has the financial incentive or organizational capacity to use. Vermont's Oliver Wyman analysis found this pattern explicitly: VITL (Vermont's health information exchange) exists but is not widely used, OneCare (Vermont's ACO) exists but participation is voluntary and incomplete, and the 'all-payer' system does not actually include all payers. The infrastructure exists; the incentives and operations to use it effectively do not.

The six pillars interact through a logic that this book develops in detail in Chapter 1\|. The short version is this: payment reform (Economics pillar) changes what behaviors the financial system rewards; clinical redesign (Clinical pillar) changes how care is actually delivered; technology investment (Technology pillar) provides the data infrastructure that makes new care models manageable at scale; policy and regulatory change (Policy pillar) creates the mandatory architecture that ensures voluntary actors cannot opt out of structural reform; equity investment (Equity pillar) ensures that transformation benefits reach the populations that need them most and does not concentrate gains among those already well-served; and operational execution (Operations pillar) converts strategy into implemented programs, trained staff, and measured outcomes.

Remove any one pillar and the transformation fails. Vermont's reform agenda is instructive precisely because Acts 167 and 68, the AHEAD Model, and the Rural Health Transformation Program together address all six pillars simultaneously — imperfectly and incompletely, as any real-world policy effort does, but with a comprehensiveness that distinguishes Vermont's approach from single-pillar interventions.

A Note to the Reader — How to Use This Book

The Four Reader Profiles

This book has four primary reader profiles. Each has a different relationship to the material and a different way of getting maximum value from the book.

The Healthcare Policy Professional

You work in a government agency, a legislative office, a think tank, or a foundation focused on healthcare policy. You have substantive policy knowledge but may not have deep familiarity with the operational mechanics of healthcare transformation — what it actually takes to implement a global budget, build a community health team, or manage a 14-hospital transformation planning process.

Recommended reading path: Read the Preface and Introduction in full, then Chapter 1 (six-pillar framework) and Chapter 2 (The Execution Sequence), with Appendix A for the full Vermont system portrait. For each pillar you care most about, read the corresponding chapter in depth. Read Chapter 16 (AHS Restructuring) carefully — it is the most direct bridge between policy mandate and organizational design. Read Chapter 15 (future forecast) for the national context.

The Healthcare Executive or Administrator

You lead a hospital, health system, health plan, ACO, or healthcare services organization. You are making decisions — about contracts, capital investments, workforce, technology — that the transformation environment is complicating. You need the analytical framework but you especially need the operational implications.

Recommended reading path: Read the Introduction for the overarching argument, then go directly to the chapters most relevant to your immediate decisions. If you are navigating a VBC contract, Chapter 6 (Economics) is your entry point. If you are building care management programs, Chapter 10 (Clinical). If you are evaluating technology investments, Chapters 12 and 13 (Technology). Read Chapters 7 and 8 (Operations) for the hospital transformation planning framework. Return to Chapters 1-3 for the integrating logic.

The Vermont Practitioner

You work for AHS, GMCB, DVHA, the Vermont legislature, a Vermont hospital, a Blueprint practice, or a Vermont community organization. You are living inside the transformation this book describes. You need the national analytical context but you especially need the Vermont-specific operational guidance.

Recommended reading path: Read Appendix A (Vermont System Portrait) for system context, then Chapter 2 (The Execution Sequence) and Chapter 3 (Policy — Acts 167 and 68) first, since these are the chapters most directly grounded in Vermont's current situation. Then read Chapter 16 (AHS Restructuring) as the operational framework for the work you are doing. Use the implementation timeline tables throughout Part II as a reference guide to where Vermont is in each pillar's trajectory. The Vermont in Practice sidebars throughout are written for you specifically.

The Healthcare Student or Researcher

You are building foundational knowledge of healthcare transformation, either in a graduate program or as an independent researcher. You need the analytical architecture, the evidence base, and the theoretical frameworks, not just the Vermont case study.

Recommended reading path: Read the entire book in chapter order. Chapter 1 establishes the framework; Chapters 3-13 develop each pillar with full academic grounding before the Vermont application; Chapters 14-16 provide the advisory infrastructure, future context, and AHS restructuring roadmap. The Key Concepts sections at the end of each chapter are the glossary; the source notes are the bibliography. Use Vermont as the case study while developing transferable analytical skills.

The Vermont Thread: How It Runs Through the Book

Vermont appears in this book in four distinct registers, and recognizing which register you are reading helps calibrate how to use the material.

- Vermont as diagnostic case: The Oliver Wyman analysis of Vermont's hospital financial crisis, the GMCB price data, and the AHS demographic projections establish Vermont as an instance of a generalizable pattern of rural healthcare system stress. When these appear, they are teaching the pattern, not just Vermont's version of it.
- Vermont as legislative laboratory: Acts 167, 51, 68, and 62 are specific legislative interventions in a specific state, but their design logic — the reform cascade structure, the mandatory versus voluntary distinction, the statutory accountability metrics — is generalizable to any state undertaking comparable reform.
- Vermont as operational field report: The AHS transformation reports, the RHRC engagement methodology, the 14-hospital planning process, and the Health Care Delivery Advisory Committee's work are real-time accounts of a transformation in progress. They convey the texture of execution — the difficulties, the adaptations, the gap between strategy and implementation — that no policy analysis can substitute for.
- Vermont as national template: Chapter 16 (AHS Restructuring) and Chapter 15 (The Future) explicitly use Vermont as a template for the national condition. Here the argument is prospective: if Vermont succeeds, this is what success looks like; if Vermont faces these organizational design challenges, every state will face them; if Vermont's approach to the AHS restructuring mandate works, other states can adapt it.

The HTR Platform and This Book

This book is part of the transformation knowledge ecosystem. The technical infrastructure framework — its intelligence feed, Research Lab tools, Academy curriculum, and Advisory services — provides the operational complement to the analytical framework the book establishes. Understanding the relationship between the book and the platform helps readers get maximum value from both.

The book provides the analytical framework: the six-pillar architecture, the evidence base for each pillar's mechanisms, the Vermont case study, and the conceptual vocabulary for understanding transformation. The HTR platform provides the operational tools: the APM Shared Savings Calculator that applies the Economics pillar's financial modeling to specific contracts; the Clinical Quality Optimizer that implements the Clinical pillar's HEDIS improvement methodology for specific organizations; the Health Equity Studio that operationalizes the Equity pillar's disparity analysis for specific patient populations; the AI Analyst that retrieves and synthesizes the framework intelligence on any topic in real time.

The Advisory services described in Chapter 14 are the institutional bridge between the book's frameworks and specific organizational decisions. Every concept in this book exists at the intersection of understanding and doing — grasping the framework clearly enough to explain it, and translating the framework into a specific decision, program, contract, or investment in a specific organizational context. The book handles the first challenge; the Advisory services handle the second.

A Note on the AHS Restructuring Chapter

Chapter 16 — The AHS Restructuring Roadmap — occupies a unique position in this book. It addresses the Statutory Mandate embedded in Act 68, conducts an Organizational Diagnosis of why AHS must restructure rather than merely plan, and provides a six-pillar framework for the 2028 Statewide Health Care Delivery Strategic Plan. It is written for the AHS staff, Health Care Delivery Advisory Committee members, Vermont legislators, and hospital leaders who must execute that mandate.

If you are a Vermont practitioner, Chapter 16 is written for you directly. It is designed to be usable as a working document — a framework you can bring to a planning meeting, a governance structure you can adapt for your organization, a metrics framework you can reference when building the Strategic Plan's accountability architecture.

If you are not a Vermont practitioner, Chapter 16 is still important reading, because the organizational design questions it addresses — how does a state agency restructure from program administration to population health system operation, how do you align a multi-program organization around geographic service areas, how do you build the analytics capacity for strategic decision-making when that capacity does not currently exist — are questions every state will face. Vermont's answers may not be directly transferable, but the analytical framework for thinking through the answers is.

What This Book Cannot Do

Intellectual honesty requires stating what this book cannot do, alongside what it can.

This book cannot predict whether Vermont's transformation will succeed. The statutory mandates are in place. The capital is flowing. The organizational machinery is in motion. But the outcome depends on thousands of decisions that will be made by hospital executives, primary care physicians, community health workers, legislators, regulators, and patients over the next five to ten years. This book provides the analytical framework for those decisions; it cannot make them.

This book cannot substitute for the operational expertise required to execute transformation. The frameworks in this book are accurate and the evidence base is solid, but implementing a global budget requires actuarial expertise, regulatory knowledge, and operational experience that a book cannot provide. That is what Advisory services, technical assistance organizations, and experienced practitioners are for.

This book cannot resolve the political contests that surround healthcare transformation. Reference-based pricing is opposed by hospital systems that benefit from high prices. Global budgets are feared by hospitals that worry about access restrictions. AHS restructuring is resisted by programs that do not want to be reorganized. These are legitimate conflicts of interest, and they will not be resolved by analytical frameworks. They will be resolved by political processes — legislative deliberation, regulatory proceedings, stakeholder negotiations — that this book can inform but cannot replace.

What this book can do: provide the analytical architecture for understanding what is happening and why; supply the evidence base for evaluating what is working and what is not; develop the conceptual vocabulary for communicating clearly across the disciplines — clinical, financial, policy, technological, operational — that transformation requires; and give Vermont practitioners the framework to see their specific work in the context of a coherent transformation agenda. That is enough to be worth writing, and worth reading.

Sources for Chapters 1-3: Oliver Wyman Healthcare and Life Sciences Group, Act 167 Community Engagement: Recommendations (August 2024); Vermont Agency of Human Services, Vermont Rural Health Transformation Program Application (November 2025); Vermont RHT Program Activity Details (December 2025); GMCB VHCURES Data and Analytics documentation; KFF State Health Facts (Vermont); Vermont Department of Health; Act 68 of 2025; Act 167 of 2022; AHEAD State Agreement (January 2025); all sources cited in subsequent chapters.